



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Sexual safety in Mental Health

Reference #: FSFH-MENH-POL-0012

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service	Medical, Nursing, Allied Health

1. Introduction

Consumers accessing mental health services may be exposed to potential risks, particularly when they are acutely unwell. The Mental Health Service recognises that consumers have the right to feel and to be safe. The purpose of this policy and procedure is to support the Mental Health Service in meeting its responsibility in relation to maintaining the sexual safety of all mental health consumers, families, carers, staff and others accessing the services.

2. Terminology

Sexual safety	Refers to being and feeling psychologically, emotionally and physically safe, including being free of, and feeling safe from behaviour of a sexual nature that is unwanted, or makes another person feel uncomfortable, afraid or unsafe.
Sexual assault	A broad term used to describe a range of sexual acts and behaviours committed against a person without their consent. Sexual assault is a crime of violence where a person uses their power and control to dominate and/or intimidate another.
Sexual harassment	Can occur between any combinations of individuals and is defined by the Australian Human Rights Commission as 'unwelcome conduct of a sexual nature which makes a person feel offended, humiliated and/or intimidated where that reaction is reasonable in the circumstances.' It can involve physical, visual, verbal or non-verbal contact.
Consensual sexual activity	Any activity of a sexual nature (touching, intercourse, oral sex, kissing) that occurs between people over the age of 16 years after mutual consent has been provided by those involved, who have been assessed as having the capacity to consent.

Sexually disinhibited behaviour	Poorly controlled behaviour of a sexual nature, where sexual thoughts, impulses or needs are expressed in a direct or disinhibited way, such as in inappropriate situations where it is socially or contextually inappropriate. This behaviour can arise for a variety of reasons that can include a mental illness or disorder.
Inappropriate context or setting	This refers to sexual activity between consumers, or consumers and their visitors or consumers and staff; taking place in an environment or associated with a set of circumstances that is unsuitable, such as: • an acute inpatient setting • public areas • Outpatient/non acute setting
Child sexual abuse	Occurs when an adult, adolescent or child engages a person under the age of 16 years in sexual activity.
Trauma informed Care	Acknowledges most people who access mental health services have experienced some form of trauma in their lives and proactively engages recognising, understanding and accommodating, including providing physical, psychological and emotional safety to actively minimise the risk of re-traumatisation.
Gender	Gender refers to the socially constructed roles, behaviours, expressions and identities of girls, women, boys, men, and gender diverse people. It influences how people perceive themselves and each other, how they act and interact, and the distribution of power and resources in society. Gender is usually conceptualised as a binary (girl/woman and boy/man) yet there is considerable diversity in how individuals and groups understand, experience, and express it ¹. A person's sex and gender may not necessarily be the same. An individual's preferred gender may or may not correspond with the sex or gender assigned at birth and some people may identify as neither male nor female ².
Gender sensitivity	Practice that recognises and responds to the differences, inequalities and specific needs across the gender spectrum.

3. Policy

The Mental Health Service will:

- Promote consumer rights throughout the service
- Support consumers, their families and staff to adopt practices and behaviours that contribute to their sexual safety, both within the mental health service environment and the broader community.

- Promote and model positive relationships and mutual respect between staff; between staff and consumers; between staff and carers/family members/guardians; and between consumers themselves.
- Create a culture of acceptance that normalises diverse sexual orientations and gender identities to assist in promoting sexual safety as well as encouraging openness in reporting sexual safety incidents ²
- Provide and make readily accessible throughout the service clear information on individual rights, acceptable and unacceptable behaviours and processes for reporting or registering a complaint regarding issues of sexual safety to consumers, carers/families and advocates³.
- Ensure all consumers have a screening for Family and Domestic Violence completed on admission to the service.
- Take appropriate action to prevent sexual safety incidents ³.
- Ensure disclosures from consumers about incidents that compromise or breach their sexual safety are taken seriously and addressed appropriately and promptly with regard for the consumer's privacy, dignity, past trauma, cultural background, gender, religion, sexual identity, age and illness. It is the responsibility of all staff to take timely affirmative action in this instance refer to section 4.4.
- Ensure all staff including volunteer's complete criminal record screening prior to the commencement of employment.

It is illegal for a staff member to engage in sexual activity with a consumer. Refer to the WA Criminal Code.

4. Procedure

4.1. Environment

- The Mental Health Service will provide a sexually safe environment which is:
 - Trauma informed; based on the principles of safety, trustworthiness, choice, collaboration and empowerment ⁵. and emphasises recovery.
 - Fair and equitable, available to a diverse range of individuals and free from discrimination.
 - Compassionate, sensitive and promotes a culture that encourages reporting of sexual safety incidents relating to consumers, family/carers/guardians and staff.
- Care provided will be safe and non-judgemental for LGBTIQ+ consumers through:
 - actively promoting the service's acceptance of diverse sexual orientations and gender identities.

- asking how consumers identify in relation to gender and the identity terms, names and pronouns that they use and document in their medical records.
 - using language that is gender neutral.
- The Mental Health Service (MHS) will balance personal autonomy and decision making with a duty of care to provide a safe and therapeutic environment for all consumers, personal support persons, visitors and staff.
- Prior to admission patients and their Personal Support Person (PSP) will be provided with information about the inpatient mental health setting – whenever possible.
- On admission to the service consumers will be provided with an orientation to the inpatient/community setting using the MH Admission Checklist that includes:
 - their right to a safe and therapeutic environment
 - an explanation that sexual behaviour and activity on the unit/service is inappropriate and not permitted
 - that intimate behaviour or sexualised contact with other consumers or staff is not acceptable.
- The daily patient meeting in the inpatient setting will discuss Mutual Expectations regarding Sexual Safety.
- Consumers will be informed that taking photographs/videos whilst on an inpatient unit is not appropriate, may breach other consumers privacy.
- Gender specific bathrooms and toilets will be provided in all inpatient areas.
- Staff allocation will consider consumer preference, clinical need and staff availability. Unless clinically contraindicated consumers will be asked for staff gender preference. Consumers will be informed how to access their allocated nurse.
- Staff rostering will consider gender ratios where appropriate.
- Staff are encouraged to:
 - recognise the consumers need for privacy and personal space, and that these needs vary between individuals regardless of gender or gender orientation.
 - complete the Sexual Assault Resource Centre (SARC) online training module available on the Mental Health Hub page.

4.2. Assessment of sexual safety risk

- Risk formulation will be completed on admission to the mental health service and documented in the consumers digital medical record (DMR).

Types of behaviour that breach and/or compromise the sexual safety of a mental health consumer may include:

- Sexual assault or harassment

- Consensual sexual activity in an inappropriate context/setting
- Sexually disinhibited behaviour.

- Assessment of a consumer's risk of experiencing a breach of their sexual safety⁴ may include:

Factors identified as placing consumers at higher risk	
Personal characteristics	• female, • identifying as LGBTIQ+ • from Culturally and linguistically diverse (CaLD) background, • being Aboriginal • having a disability • older or younger
Trauma history	
Vulnerability associated with mental illness	• experiencing psychosis • intellectual disability • impaired communication skills • heavily sedated, intoxicated and/or having co-occurring alcohol and other drug conditions • distress and social isolation
Capacity to make informed decisions	

- This assessment will inform the formulation of the Risk Assessment Management Plan (RAMP) along with other risks such as risk to self and others, risk of violence, physical health risk or other vulnerability

4.3. Treatment and support

- Consumers will be encouraged to be involved in ward activities and groups to minimise isolation and risk of harm from others
- Visitor interactions will be monitored, any inappropriate behaviour, particularly of a sexual nature will be reported immediately.
- Use of personal electronic devices will be regulated where appropriate.

- To ensure a sense of sexual safety during an inpatient admission the consumer will be engaged in safety planning and protective behaviours education in collaboration with staff.
- The consumers Treatment Support and Discharge Plan (TSDP) documented in the DMR will include:
 - how the consumer identifies in relation to gender and sexuality
 - consumer preferences regarding the gender of clinicians
 - reported concerns regarding other consumers and how their concerns are to be managed
 - any identified sexual safety issues/concerns and actions taken to address them
- Consumers noted to be displaying sexualised behaviours and/or at-risk consumers will be highlighted during clinical handover and discussed in multidisciplinary (MDT) team review meetings. The team will identify support and management strategies such as:
 - plans to manage sexual disinhibition
 - ensure maximum visibility of consumers by allocating bedrooms closest to the nursing station
 - visual observations for consumers at risk of perpetrating inappropriate sexual conduct and victimisation
 - two staff, at least one of the same gender are to be present during routine hourly safe checks at night.
 - visual observations of consumers will be initiated, maintained and documented in line with the FSFHG Patient Observation – Specialising and Close Observations policy and procedure.
 - offered discreet, non-intrusive and non-custodial measures such as providing feedback about acceptable behaviour and/or providing a temporary location that provides safety for all.
- In community-based services clinicians will:
 - identify any sexual safety issues that may arise in the Outpatient Clinics or when visiting the consumer in their home
 - discuss with the MDT and document in the DMR and RAMP where appropriate
 - document the management plan in the TSDP
 - place alerts in the Psychiatric Services Online Information system (PSOLIS) when appropriate.
 - comply with the SMHS Community/Home visiting policy

4.4. Sexual safety incidents

- Where a consumer identifies a sexual safety breach staff will:

- assess the consumers immediate safety and wellbeing and need for medical treatment.
 - ensure the consumers right to privacy and confidentiality are respected
 - escalate reported sexual breaches to immediate line manager and Consultant Psychiatrist
 - identify and document priority actions and safety planning to increase sexual safety in consultation with the MDT
 - provide appropriate support/treatment and or referral to services e.g. Sexual Assault Resource Centre (SARC), Police
 - document in both the DMR and the Datix Clinical Incident Management System (CIMS) and report all sexual safety breaches including reports of alleged sexual assault, harassment, sexual activity, sexually disinhibited behaviour.
 - consider language in the medical record if it may impact on consumer management
 - update Risk Assessment and Management Plan and Treatment Support Discharge Plan
- Place an alert on PSOLIS for patients with known/reported offences as perpetrator
- Follow the guidelines outlined in:
 - DoH Responding to an Allegation of sexual assault disclosed within a Public Mental Health Service
 - Mandatory reporting of notifiable incidents to the Chief Psychiatrist.

4.5. Staff safety

- Mental health staff have the right to feel safe while in the mental health inpatient environment or in the course of their work in the community.
- Community visits will be planned after considering:
 - consumer / staff gender
 - location of visit
 - PSOLIS alerts
 - Professional Code of conduct
 - previous history of boundary violations with previous staff
 - history of false allegations or complaints of a sexual nature
- Sexual safety concerns related to community visits will be discussed with the team and two staff members to visit will be considered to mitigate risk as appropriate
- Staff in all settings will be provided with optimal opportunity to attend training on managing sexual safety, trauma informed care and reporting of sexual safety incidents.

- Provide staff with the opportunity to debrief and /or access to the Employee Assistance Program.

5. Compliance/Performance Monitoring

The Mental Health Service will conduct an annual sexual safety audit to assess sexual safety within the inpatient setting. The audit report will be tabled at the Safety Quality Risk Committee.

The Mental Health Service submits data to the Australian Council of Health Care Standards Mental Health Clinical Indicators six monthly. Clinical Indicator reports will be tabled and monitored by the Safety Quality Risk Committee.

6. Related Policy Documents

WA Health:

- [Policy for Mandatory reporting of notifiable incidents to the Chief Psychiatrist](#)
- [Chief Psychiatrists Guidelines for the Sexual Safety of Consumers of Mental Health Services in WA](#)
- [Responding to an allegation of sexual assault within a public mental health service policy](#)
- [Code of Conduct MP0124/19](#)
- [Complaint management policy Mp0130/20](#)
- [Clinical Incident Management Policy MP 0122/19](#)
- [Statewide Standardised Clinical Documentation for Mental Health Services MP0155/21](#)
- [Mental Health Act 2014 Charter of Mental Health Care principles](#)
- Guidelines for Protecting Children 2015 (Revised 2017)

SMHS:

- [Community/home visiting](#)
- [Rights of patients in mental health](#)

FSFHG:

- Clinical Risk Assessment and Management (FSFH-HW-POL-0017)
- Clinical Handover (FSFH-HW-POL-0035)
- Patient Observation – Specialising and Close Observations (FSFH-MENH-POL-0023)
- Mobile phone use Youth Unit (FSFH-MENH-GUI-0001)

7. Related documents

Mental Health Act 2014

Criminal Code Act 1913

8. Related Standards

NSQHS Standards:

- Clinical Governance
- Partnering with Consumers
- Communicating for Safety

Chief Psychiatrists Standards for Clinical Care:

- Care planning
- Consumer and carer involvement in individual care
- Risk assessment and management

9. References

1. Canadian Institute of Health Research. Definitions of Sex and Gender. 16/09/2019]; Available from: <http://www.cihr-irsc.gc.ca/e/47830.html>.
2. Chief Psychiatrists Guidelines for the Sexual Safety of Consumers of Mental Health Services in WA
3. New South Wales Government. (2013). *Sexual Safety of Mental Health Consumers Guidelines*. NSW Ministry of Health. New South Wales. Australia
4. Mental Health Coordinating Council. (2019). *Trauma-informed Care and Practice Organisational Toolkit*. New South Wales. Australia. NSW.www.mhcc.org.au
5. NSW Ministry of Health, Sexual Safety of Mental Health Consumers Guidelines, Mental Health Drug and Alcohol Office, Editor. 2013, NSW Government: North Sydney NSW.

10. Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	09/2016	CNS, Clinical Improvement	FH MHS CGC	FSFHG Policy Committee	09/2019
2	09/2019	CNS, Clinical Improvement	NMPGC	FSFHG Policy Committee	09/2022
3	11/2022	CNS, Clinical Improvement	NMPGC	Service 5 Safety Quality and Risk Committee	07/2026
4	02/2024	CNS, Clinical Improvement	NMPGC	Service 5 Safety Quality and Risk Committee	07/2026